Centers for Medicare and Medicaid Services

Health Technology Ecosystem

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Glossary of Abbreviations and Acronyms

Acronym	Description
ACO	Accountable Care Organization
AI	Artificial Intelligence
API	Application Programming Interface
BDV	Big Data Virtualization
CBO	Community-Based Organization
CMS	Centers for Medicare & Medicaid Services
DHA	Defense Health Agency
FFS	Fee-for-Service
FHIR	Fast Healthcare Interoperability Resources
GENOME	Automated Legacy Code Refactoring Engine
HL7	Health Level Seven International
IHS	Indian Health Service
LDCS	Legacy Data Conversion Solution
LTSS	Long Term Services and Supports
MA	Medicare Advantage
MDACA	Multiplatform Data Acquisition, Collection, and Analytics
MIDAS	Multidimensional Insurance Data Analytics System
MHS	Military Health System
MIP	MHS Information Platform
OMOP	Observational Medical Outcomes Partnership
PII	Personally Identifiable Information
SDE	Synthetic Data Engine
SDK	Software Development Kit
SDMA	Simple Data Mesh Architecture
VA	Department of Veterans Affairs
VBC	Value-Based Care

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1.0 Introduction

SpinSys-Diné is pleased to submit the following response to the Centers for Medicare & Medicaid Services (CMS) Request for Information on the Health Technology Ecosystem. We attended the CMS Health Technology Ecosystem webinar, opened by Dr. Oz, and appreciated the opportunity to hear CMS's priorities and vision for this effort. As directed during the session, we are providing responses to those questions where our experience and capabilities can offer the most relevant insight and practical support for CMS's goals.

Why Spin-Sys-Diné

Proven Federal Contracting Expertise 20+ years of successful prime contract execution and task order (TO) delivery, backed by deep functional and technical expertise across $350M+ in acquisitions

Small Business Agility with Enterprise Strength
Currently managing $175M+ annually in DoD contracts. Rapid response capabilities with the support of a well-established corporate Program Management Center of Excellence and Shared Services Center, including Innovations, Quality Management, Contracts, Proposals, and other key operational teams

"Team of Teams" Model
Used during the AMP project, or model accelerates mission success, reducing onboarding time by 50%, cutting operational costs by up to 30%, and enhancing system efficiency by 40% through seamless integration of specialized expertise, automation, and optimized cloud and data services, driving agility, cost savings, and future-ready innovation

SpinSys-Diné brings two decades of direct support to the Military Health System and the Defense Health Agency (DHA), delivering secure, resilient IT services at enterprise scale. We understand that your mission is not just technical. It is operational, medical, and strategic. We help clients improve interoperability, reduce provider burden, enable safe AI-driven innovation, and advance value-based care outcomes. SpinSys-Diné a subsidiary of Diné Development Corporation (DDC), established in 2004 and wholly owned by the Navajo Nation, is a tribally owned enterprise uniquely positioned to support federal agencies by combining the agility of a small business with the robust capabilities of a large corporation. Our extensive expertise spans Cybersecurity, Health IT, Cloud Computing, Artificial Intelligence, Data Management, Agile Development, System Integration, and Enterprise Digital Transformation. Differentiators to our approach are the access granted to our DDC Shared Services Center and Strategic Innovations Team. Our Shared Services Center is a centralized hub that integrates cutting-edge innovations, industry best practices, and specialized expertise across all our subsidiaries. This resource enables us to provide seamless access to enterprise-grade capabilities from all our subsidiaries while maintaining the responsiveness required for rapid innovation and evolving mission requirements. Our Strategic Innovations Team leverages AI powered automation, predictive analytics, and cybersecurity best practices, enhancing operational efficiencies. Our tools and approaches can empower CMS to foster innovation, improve patient experience, and drive digital transformation at scale.

Through the broader network of DDC subsidiaries, CMS will have access to an extensive portfolio of contract vehicles, including General Services Administration (GSA) Multiple Award Schedule (MAS), GSA One Acquisition Solution for Integrated Services (OASIS+), GSA Streamlined Technology Acquisition Resource for Services (STARS III). Additionally, some of our subsidiaries are eligible for Tribal 8(a) sole-source awards under 13 CFR § 124.506(b) and Defense Federal Acquisition Regulation Supplement (DFARS) 206.303-1(a). The multitude of acquisition pathways enables the government to accelerate acquisition timelines, minimize procurement risks, and leverage a highly experienced, cost-effective partner with a proven record of accomplishment in delivering DOD healthcare IT solutions.

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2.0 Company Information

Company Name:	SpinSys-Diné
GSA Schedule/GWAC Contract Number(s):	GSA MAS GSA OASIS+ Available through Subsidiaries: GSA 8(a) STARS III Polaris CIO-SP4
SAM.gov UEI:	MMUTKX7AUN65
NAICS Codes:	541330 (primary), 541512, 511210, 518210, 541330, 541511, 541513, 541519, 541611, 541330, 611420
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Business Size:	Small
Business Classification:	Minority-Owned Business, Self-Certified Small Disadvantaged Business, Native American Owned
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Company Phone:	(703) 318-0803 x236
CAGE Code #:	9ML17
SAM.gov UEI:	MMUTKX7AUN65

3.0 Responses to Government Questions

PC-1 What health management or care navigation applications would help you understand and manage your (or your loved ones') health needs, as well as the actions you should take?

SpinSys-Diné believes CMS can make significant progress by supporting applications that simplify access to care, improve navigation of services, and reduce burdens on caregivers and families. Today's healthcare system often feels fragmented and overwhelming. Many patients do not know what coverage they have, what actions they should take, or how to make informed decisions. This is especially true for older adults, individuals with chronic conditions, or those managing care for a loved one.

Patients and caregivers would benefit from tools that provide proactive, contextual guidance. This includes digital health products that consolidate benefits, claims, and clinical records; suggest next best actions; and follow up on care plan adherence.

a. What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

CMS could expand access to tools that enable:

- **Personalized care navigation**: A single digital view showing available services based on coverage, history, and preferences. This includes what is covered under Medicare or Medicaid and recommended next steps.

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- **Benefits comprehension and decision support**: Tools that explain coverage in plain language, suggest plans during open enrollment, and alert users when key benefits go unused (e.g., annual wellness visits or preventive screenings)
- **Care coordination visibility**: For people seeing multiple providers, a centralized platform could show recent visits, medications, care plans, and follow-up tasks. Caregivers could use this same view to track progress, appointments, and care gaps.
- **Medication and condition tracking**: Simple tools that help users keep up with prescriptions, symptoms, vitals, or chronic condition trends, and easily share updates with providers or family
- **Secure communication with care teams**: A platform for questions, updates, and guidance between visits, supporting both patients and authorized caregivers

We've seen that users value simplicity. The best tools don't try to do everything. They focus on solving the everyday problems of people trying to live their lives while staying healthy or caring for someone who needs support.

b. If you had a personal assistant to support your health needs, what are the top things you would ask them to help with?

If people had access to a trusted digital health assistant, they would likely ask it to:

- **Remind them of important appointments or tests** based on their age, history, or conditions
- **Explain new diagnoses, benefits, or letters** in language they understand
- **Help schedule care or connect with providers**, including telehealth
- **Flag changes in coverage, eligibility, or costs**, especially during plan renewals
- **Track symptoms or health goals**, with nudges to act when something looks off
- **Keep family in the loop** by sharing updates with approved caregivers or caseworkers
- **Provide local resource recommendations**, such as transportation, food assistance, or community-based services

Many of these needs have been met in isolated ways across states, plans, or startups. CMS could lead by creating or supporting a national framework for these capabilities, especially for those with the most complex needs.

How SpinSys-Diné Can Support CMS

SpinSys-Diné could help CMS accelerate transformation by bringing proven solution accelerators, reusable components, and delivery patterns shaped through work across the Defense Health Agency (DHA), Indian Health Services (IHS), and the Department of Defense (DoD). These capabilities are not conceptual; they've already reduced time-to-impact in federal health programs with millions of beneficiaries.

Here's how we could apply them to CMS's goals:

1. Digital Health Navigation Capabilities

Tested in DHA environments, this digital front end unifies health actions, benefits, and coverage views across siloed systems. It's built on open standards and integrates with CMS APIs and FHIR-based backends. We could adapt it to help Medicare and Medicaid beneficiaries complete wellness visits, manage prescriptions, or understand discharge instructions.

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2. Family and Caregiver Coordination Features

We've delivered shared-care toolkits through the Military Health System (MHS) Information Platform (MIP) and IHS Resource and Patient Management System (RPMS). These support multi-user access with configurable consent management, task tracking, and care calendars. These are key needs for dual-eligible populations and Long Term Services and Supports (LTSS) care models. We could tailor this toolkit to CMS's caregiver engagement strategies.

3. Conversational Guidance Functions

Based on our deployments with Veteran Affairs (VA) and DHA, a rules-driven assistant could interpret benefits, clinical timelines, and user intent. It could explains next steps in plain language via voice or text and can be configured to CMS policies. We could use this to support high-friction events like denied claims, appeals, or open enrollment.

4. De-Identified Behavioral Signal Library

SpinSys-Diné has developed synthetic behavioral data patterns to simulate navigation behavior, bottlenecks, and benefit usage without exposing PII. CMS could use this to validate new digital services (e.g., family health dashboards or proactive outreach tools) before scaling.

5. Interoperability Standards Implementation

We've operationalized HL7, FHIR, and OMOP across DoD, VA, and CMS analytics platforms, including the ingestion of 1.7PB into the MIP while processing over 9.5B transactions daily. These patterns support real-time clinical exchange and analytics. CMS could use this foundation to unify Medicare data across MA plans, fee-for-service, and CBOs.

6. Multiplatform Data Acquisition, Collection, and Analytics (MDACA) and Patient-Facing Digital Tools

Patients benefit from proactive, contextual guidance, especially through mobile and voice-enabled tools. We've integrated claims, benefits, and clinical records into digital assistants that surface next-best actions, explain results, and support adherence. Our MDACA platform brings together patient-generated data and predictive analytics, enabling CMS to offer personalized services through platforms like MyMedicare.gov or its app equivalents.

PC-3 Are you aware of health management, care navigation, or personal health record applications that would be useful to Medicare beneficiaries and their caregivers?

Yes, we're aware of several health management, care navigation, and personal health record applications that could offer value to Medicare beneficiaries and their caregivers. Most fall short in one or more key areas: accessibility, integration, personalization, or trust. Many tools work well in silos but do not operate at the scale or complexity CMS requires. We believe CMS has an opportunity to lead in bringing these tools together into a coherent, user-centered experience that builds trust, supports equity, and works across all Medicare programs.

Existing Applications with Relevance:

1. **Blue Button 2.0 and MyMedicare.gov**: These tools provide access to claims data, but lack actionable guidance. Most beneficiaries do not know how to use them to manage care or make decisions.

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2. **Apple Health and CommonHealth**: These applications aggregate records using FHIR APIs. However, they assume digital literacy and access to compatible provider systems. Adoption among Medicare populations remains limited.

3. **AARP Caregiving App**: Offers shared calendars, task lists, and care planning. Useful for caregivers, but not linked to CMS data or benefits.

4. **Medisafe and MyTherapy**: Medication reminder tools with engagement features. These address part of the burden but don't connect to Medicare coverage or care coordination.

5. **Veterans Affairs' MyHealtheVet**: A more mature platform supporting secure messaging, lab results, appointments, and prescription refills. Its depth and integration could serve as a model for CMS.

How SpinSys-Diné Can Support CMS

While these tools show value, they each address only part of the user's experience. Most require the user to already know what to do and how to find it. That's the gap CMS could fill. SpinSys-Diné could help CMS build a more complete experience by applying insights and technical assets from our prior work. These include:

1. Personal Health Record Integration Patterns

We assisted DHA design unified health data views across multiple systems. We've worked with HL7, FHIR, and custom data models to normalize and expose key health events for individuals and families. We can help CMS adapt those patterns to integrate claims, encounter, and care plan data into a trusted Medicare health record. This record could power downstream services like medication tracking, upcoming care actions, or care team coordination.

2. Secure Caregiver Access Framework

We've delivered caregiver and role-based access models through MHS and IHS systems. These included support for delegation, audit logging, and secure messaging across military, family, and civilian contexts. We can help CMS implement flexible caregiver support, including shared access to personal health records, appointment tools, and educational content.

3. Navigation Layer for Care Actions

Through our work on GENOME and SDE, we've built logic engines that interpret data and surface recommended actions. In previous programs, this has included:

- Next steps after discharge
- Gaps in preventive care
- Missed medication refills
- Changes in cost sharing

We can apply these solution accelerators to create a CMS care action layer that adapts to user needs, preferences, and risk factors.

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4. Interoperability with Community-Based Tools

SpinSys-Diné has helped government clients map and exchange data with public health, social service, and tribal care programs. That experience could support CMS integration with community navigators, state Medicaid agencies, and other local networks often accessed by Medicare-Medicaid duals.

While many applications exist today, we believe CMS can lead by building an integrated platform focused on health literacy, shared decision-making, and digital equity. We can bring proven tools, delivery experience, and stakeholder empathy to support that vision.

PC-4 What features are missing from applications you use or that you are aware of today?

Many existing health management applications fall short because they assume users already understand their healthcare, their benefits, and what steps to take. These tools often miss the real-world needs of older adults, caregivers, and people with complex or changing health situations. They also tend to ignore the barriers that many CMS beneficiaries face such as low digital literacy, chronic illness, multiple providers, or limited support.

a. What applications should exist but do not yet? Why do you believe they do not exist yet?

1. Integrated Care + Coverage View

Most tools separate medical care from health benefits. For example, someone might know they have diabetes, but not whether their Medicare Advantage plan covers a new glucose monitor or what paperwork is needed to get it. What's missing is a tool that shows users:

- What condition(s) they have
- What services are covered
- What's recommended based on current guidelines
- What action(s) to take today

This doesn't exist because of data silos. Providers control clinical data. Plans manage coverage data. CMS holds claims. No tool today can bridge all three without full policy and data support from CMS.

2. Shared Navigation for Families

Families rarely have a single person managing care. It's often a combination where one person handles appointments, another handles insurance, and a third provides in-home help. Tools that allow this kind of shared care management, with secure delegation and different views for different roles, are still missing.

We believe they don't exist at scale because HIPAA compliance, state-by-state laws, and Medicare system complexity create real integration challenges. CMS is in a position to standardize this.

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3. Proactive Nudging

Applications today are reactive; they wait for users to act. What's missing is proactive nudging: reminders to schedule preventive care, alerts when medications run out, or guidance after a diagnosis. These nudges could be tied to actual claims and health history, not just generic rules.

Most commercial applications can't do this because they don't have access to CMS data. CMS is uniquely positioned to deliver this kind of targeted, trusted guidance.

4. Benefits Education That Actually Works

People don't understand what Medicare covers, how it changes each year, or what they lose if they don't act. Today's educational materials are PDF-heavy, confusing, and not interactive. CMS needs tools that explain benefits clearly, in plain language, and tailored to the user.

This hasn't happened because no private company owns the benefits rules. CMS does, which is why CMS should own this user education layer.

b. What set of workflows do you believe CMS is uniquely positioned to offer?

We believe CMS is the only organization with the authority, data, and reach to provide the following workflows in a trusted, personalized way:

1. Cross-Benefit Eligibility Checker

CMS can show a person what they qualify for across Medicare, Medicaid, Medigap, Part D, and waiver programs with clear next steps. SpinSys-Diné can help implement this by drawing from our work with eligibility engines in DHA and IHS, where we've built logic for multi-program access.

2. "What To Do Next" Engine

A workflow that connects diagnosis → benefit coverage → recommended services → provider directories → appointment scheduling. SpinSys-Diné delivered similar guided workflows across DHA platforms, using FHIR and rule-based engines to turn complex logic into user-facing actions.

3. Digital Delegation and Care Circle Management

CMS could enable secure, policy-aligned delegation of data and decision-making across caregivers and family members. SpinSys-Diné built delegation models in MIP and VA systems, tied to consent forms, audit trails, and granular access levels. We could bring those forward.

4. Personalized Plan Guidance

Every year, millions of people guess their way through plan selection. CMS could offer a guided selection tool based on usage history, priorities, and predicted needs. We have experience working with claims-based predictive models and building explainable plan comparison tools. We could help make this workflow usable, trustworthy, and human-centered.

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How we can support CMS

These capabilities do not yet exist in the market at scale because private companies lack the complete data, policy authority, or trusted position to deliver them. CMS holds all three and can lead.

SpinSys-Diné can help CMS accelerate this work by bringing forward proven, reusable components from complex federal health environments:

- **Cross-Benefit Eligibility Logic**: We have built multi-program eligibility engines for DHA and IHS, enabling a unified view across medical, pharmacy, dental, and waiver benefits. We can help CMS design and deploy a similar eligibility checker for Medicare and Medicaid populations.
- **Next Action Engines**: SpinSys-Diné has delivered guided workflows that connect diagnoses, benefits, services, provider directories, and scheduling into simple “what to do next” steps. Our Conversational Guidance Functions can support both visual and voice-first delivery of these workflows.
- **Digital Delegation and Care Circle Models**: We built secure, auditable delegation models in MIP and VA systems, with granular consent controls. We can help CMS design care circle management that respects HIPAA, state policies, and patient needs.
- **Personalized Plan Guidance**: Our team has operationalized claims-based predictive models and explainable plan comparison tools in other federal programs. We can help CMS build a guided, personalized plan selection workflow that empowers informed decisions, especially for underserved and vulnerable beneficiaries.

Across all these efforts, SpinSys-Diné can help CMS co-design inclusive workflows that address the real-world needs of patients, caregivers, and families and deliver these experiences through the trusted CMS digital ecosystem, not fragmented third-party apps.

PC-5 What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

CMS and its partners could increase patient and caregiver interest in digital health products by focusing less on features and more on trust, simplicity, and relevance. Right now, many Medicare beneficiaries either don’t know these tools exist, don’t understand how to use them, or don’t believe they’re worth the effort. Caregivers often rely on paper, phone calls, and trial-and-error because they don’t have time to explore digital alternatives that feel complex or risky. This isn’t a technology problem. It’s a trust and usability problem. CMS is in a unique position to solve it.

How CMS and Its Partners Could Encourage Adoption:

1. Endorse, Don’t Just List

CMS should publicly endorse a small set of tools that meet defined standards for usability, privacy, and accessibility. These tools should be promoted through mailers, enrollment packets, provider portals, and call centers. People trust CMS. That trust doesn’t carry over to a random app store.

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2. Require Integration into Existing Channels

The tools people already use such as MyMedicare.gov, state Medicaid portals, and plan websites should offer direct access to approved digital health tools. Don't make users start over with a new login or app. Pull trusted tools into existing workflows.

3. Provide Support for Digital Tools

We've seen in other federal programs that digital adoption improves when people can talk to someone. CMS could fund navigator programs, community health workers, or call centers to help people onboard, troubleshoot, and get value from digital products.

4. Focus on Everyday Use Cases

Digital tools should solve the real problems people face: tracking prescriptions, preparing for a specialist visit, understanding bills, or helping a parent stay independent. CMS could prioritize and highlight tools that support these tangible goals.

5. Tailor for Equity

Many beneficiaries face language, literacy, or access barriers. CMS should only promote tools that meet standards for plain language, language access, and broadband independence (e.g., offline use, SMS support, large text modes).

a. What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality, or impact?

CMS should not act as a regulator of every digital health tool, but it should define and enforce a baseline standard for tools that access CMS data, are promoted through CMS channels, or are used to inform healthcare decisions.

This review should focus on:

- **Data safety and consent**: Is user data handled in ways that match CMS rules and expectations?
- **Accessibility and usability**: Can the tool be used by older adults, those with low digital literacy, or people with disabilities?
- **Clarity and transparency**: Does the product explain how it works, who benefits, and how decisions are made?

CMS doesn't need to review every feature. But it does need to act as a trusted filter endorsing tools that meet key standards and removing access from those that don't. SpinSys-Diné can support CMS in operationalizing this review process, drawing on our work in DHA accreditation, RMF security reviews, and consumer trust evaluations.

b. What changes would enable timely access to high-quality CMS and provider-generated data on patients?

Timely access to high-quality data underpins everything from care coordination to shared decision-making. Currently, too many CMS data flows are delayed, incomplete, or trapped in legacy systems. We believe CMS could improve this by:

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1. Streamlining Consent

Enable digital, reusable consent across CMS systems. If a patient approves data sharing once to let a caregiver view medication history that consent should carry across Medicare, Medicaid, and Part D systems. SpinSys-Diné delivered similar reusable consent workflows for DoD and IHS systems and can help bring that to CMS.

2. Supporting Real-Time Access to Claims and Encounter Data

Many CMS APIs lag by weeks or months. For real-world utility, beneficiaries and providers need access to up-to-date information on services, prescriptions, labs, and referrals. CMS could pilot faster access windows for critical care episodes.

3. Prioritizing Data Provenance and Clarity

It's not enough to release data. Users and caregivers need to understand where the data came from, when it was last updated, and what it means. CMS could add labeling and summaries to claims or service records to help users understand what they're looking at.

4. Aligning Standards Across Vendors

We've seen that providers, vendors, and payers all format FHIR data differently. CMS should offer validation, reference implementations, and sample payloads to encourage consistency. SpinSys-Diné has done this work for DHA and IHS FHIR-based pilots and can help CMS accelerate these efforts.

How SpinSys-Diné Can Support CMS

CMS is trusted in a way few institutions are. That trust gives CMS the power to encourage the right tools, establish baseline expectations, and create the infrastructure for safe, timely, and useful data access.

SpinSys-Diné can support CMS in several key areas:

- **Operationalizing a trusted digital health product framework**: We can help CMS define, test, and scale a baseline evaluation process for digital tools, leveraging our experience conducting RMF security reviews, consumer trust evaluations, and accreditation processes for DHA. Our Conversational Guidance Engine and Trusted Interoperability Patterns can be used to validate that tools meet standards for transparency, accessibility, and safe data handling.
- **Building inclusive consent frameworks**: SpinSys-Diné has delivered reusable digital consent workflows for DoD and IHS. We can help CMS implement similar cross-program consent models that allow patients and caregivers to manage access preferences in clear, user-friendly ways.
- **Enabling safe developer innovation through synthetic data**: Our Synthetic Data Engine (SDE) can provide de-identified, production-like CMS datasets for testing, accelerating the ecosystem of approved tools without compromising privacy.
- **Driving consistency and trust in data presentation**: Using our experience standardizing FHIR payloads and building reference implementations for DHA and IHS, we can help CMS create validated APIs and content patterns to ensure that patient-facing apps present CMS data consistently, clearly, and with proper provenance.

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- **Supporting equity-first design**: SpinSys-Diné has built inclusive design toolkits for use in DHA, IHS, and Tribal health contexts. We can help CMS apply these patterns to digital product evaluations and ensure that endorsed tools meet the needs of diverse populations including older adults, rural residents, and those with language or literacy barriers.

PC-6 What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited digital experience?

For Medicare beneficiaries and caregivers, especially those with limited digital experience, the most important features in any digital health product are clarity, simplicity, and support. If a tool is hard to understand, confusing to navigate, or filled with jargon, people won't use it, no matter how powerful it is. We've seen this firsthand across systems serving older adults, rural populations, and caregivers juggling multiple responsibilities. When tools fail, it's rarely because of bad technology. It's because users don't trust the tool, can't figure out what it's for, or feel overwhelmed by it.

Critical Features for Accessibility and Ease of Use:

1. Plain Language Everywhere

Health tools often assume users understand clinical or insurance terms. Most don't. Interfaces must use everyday language, avoid abbreviations, and explain terms with tooltips, examples, or short videos. We've applied this in our DHA and IHS work, where users include military families, elders, and non-native English speakers. A health record isn't useful if people don't understand what it says.

2. Guided Flows with One Question at a Time

Instead of large forms or busy dashboards, show users one simple question or task per screen. Let them move at their own pace, with clear back buttons, save options, and help prompts. This approach has worked well in our user interface designs for caregiver tools and benefit applications. It reduces cognitive overload and helps people stay focused.

3. Audio and Visual Support

Allow users to click a button and hear text read aloud. Include closed captioning and visual cues (like icons or symbols) to support users with hearing, vision, or cognitive differences. These features are standard in tools we've delivered to meet Section 508 and WCAG guidelines for federal health clients. We could help CMS bake this into baseline requirements for any approved tool.

4. Multilingual Access

Many Medicare users prefer to engage in languages other than English. CMS should require that digital tools support key languages based on regional demographics. We've supported multilingual interfaces in tribal and public health systems, using open-source language frameworks and culturally adapted terminology.

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5. No Login Required (or Simplified Login)

Logging in stops many users from accessing tools. Complex passwords, device switching, and multi-factor authentication can be barriers. CMS could support passwordless login, one-time codes, or delegated caregiver access. We've implemented these approaches securely in other systems using verified identity tokens and session-based access.

6. Offline and Low-Bandwidth Functionality

Many rural and low-income beneficiaries don't have reliable internet. Tools should offer offline modes or work over basic SMS. CMS should support mobile-first design and ensure tools degrade gracefully under poor connectivity. SpinSys-Diné delivered similar features for field clinicians in DHA systems where remote work was common.

7. Clear, Trusted Branding

If a tool is connected to CMS, the user should know it. Use consistent visual language and a trust seal. People are more likely to engage when they know the tool is real and endorsed by CMS, not a scam or sales pitch. We can help CMS create a visual standard for "CMS-backed" tools, informed by our past work aligning digital products with federal branding and trust requirements.

8. Human Help on Demand

Digital products need to be paired with human support. A live help button, call-back feature, or connection to a local health navigator increases adoption, especially among users who fear "breaking something" or making a wrong choice. We've supported these workflows in prior call center and case management systems, using shared knowledge bases and real-time screen sharing support.

How SpinSys-Diné Can Support CMS

CMS has a unique opportunity to set baseline expectations for digital health products that serve all beneficiaries including those with limited digital experience. SpinSys-Diné can help CMS build and scale trusted, inclusive tools by bringing forward proven capabilities from our federal health work:

- **Plain language and multilingual design**: We have developed culturally adapted, multilingual user interfaces in tribal and public health systems, using open-source language frameworks and plain language best practices. We can help CMS define plain language and multilingual requirements for all CMS-endorsed tools.
- **Guided flows and accessible UX**: SpinSys-Diné has delivered guided, step-by-step flows in DHA, IHS, and VA systems that meet Section 508 and WCAG standards. Our design patterns reduce cognitive load and support users with a wide range of abilities. We can help CMS apply these patterns across approved digital health products.
- **Simplified login and delegated access**: We have implemented passwordless login, verified identity tokens, and caregiver delegation models in MIP and VA systems. We can help CMS enable secure, user-friendly access pathways for patients and caregivers.
- **Offline and low-bandwidth functionality**: SpinSys-Diné built mobile-first, low-bandwidth tools for DHA field clinicians and tribal public health systems. We can help

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CMS define offline-capable standards and deliver reference implementations for rural and underserved communities.

- **Trusted branding and human-centered support**: We have created visual standards and trust markers for federal health products and integrated human support features (live help, call-backs, local navigator connections) into case management and call center systems. We can help CMS design and operationalize these trust features to drive adoption and engagement.
- **AI-enhanced support and guidance**: SpinSys-Diné has implemented AI features in federal health systems to assist users with completing forms, understanding benefits, and navigating care options. We can help CMS define responsible AI patterns that extend human support, reduce call center volume, and ensure equitable access for beneficiaries with varying levels of digital literacy.

Across all these areas, SpinSys-Diné can help CMS not only define requirements but also build inclusive reference implementations, validate vendor tools, and co-design user experiences that deliver dignity, independence, and clarity for all Medicare beneficiaries and caregivers.

PC-7 If CMS were to collect real-world data on digital health products' impact on health outcomes and costs, what would be the best approach?

If CMS wants to measure the real-world impact of digital health products on outcomes and cost, the best approach is to embed measurement directly into the systems people already use such as claims processing, care coordination platforms, and provider workflows rather than trying to bolt on new reporting tools after deployment. Too many federal efforts lose visibility after a pilot ends. To avoid that, CMS could treat digital health measurement like any quality initiative: define the metrics early, tie them to routine data streams, and align with existing value-based care models.

Recommended Approach:

1. Define a Core Set of Impact Measures

CMS should publish a standard framework for measuring digital product impact across three domains:

- Health outcomes (e.g., improved control of chronic conditions, reduced hospitalizations, better adherence)
- Cost efficiency (e.g., fewer duplicative tests, reduced emergency use, avoided readmissions)
- User experience and equity (e.g., usage across income groups, rural areas, or language preferences)

These measures should build on existing frameworks like HEDIS, MIPS, and the CMS Innovation Center's Health Equity Index.

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2. Use Linked Claims and Encounter Data as the Backbone

Claims and encounter data remain CMS's most reliable source for tracking downstream outcomes. These data should be used to evaluate:

- Whether users of a digital tool follow through on recommended care
- Whether they experience improved condition management (e.g., A1C tests completed, statin use)
- Whether total cost of care is reduced over time

We've supported similar measurement strategies through DHA's MIP platform, where claims-driven outcomes are linked to digital interventions and program use.

3. Capture Structured Metadata on Tool Usage

CMS should require approved tools to submit structured logs of engagement such as duration, features used, user type (patient or caregiver), and completion of key tasks (e.g., scheduling a visit, viewing lab results). This metadata could be submitted periodically through secure APIs, using standards such as the HL7 FHIR DeviceUsage or Observation resources, allowing CMS to analyze usage patterns without violating privacy.

In addition, CMS should allow tools to integrate data from patient-authorized wearable devices (e.g., smartwatches, activity monitors, blood pressure cuffs). Wearables can provide continuous, passive signals on key health indicators such as physical activity, heart rate, or sleep which are all critical for managing chronic conditions and supporting preventive care.

4. Enable Provider and Plan Feedback Loops

CMS could also collect contextual feedback from providers and plans using the tools. This could include:

- Net Promoter Scores (NPS) from clinicians
- Reported time savings in care coordination
- Flags on duplicate or conflicting data entries

SpinSys-Diné has supported similar feedback mechanisms within Air Force and IHS systems by integrating short post-use surveys into existing workflows.

5. Partner with Academic and Community Research Networks

For deeper insights, CMS could fund research organizations to conduct prospective studies using these tools, focusing on underserved populations, dual-eligibles, or rural users. Findings could supplement CMS's quantitative data with qualitative insights. We've seen this model work in VA and DoD environments, where community and tribal partners helped evaluate the impact of tech adoption through mixed-method approaches.

6. Pilot "Data Ready by Design" Requirements

Any tool that CMS endorses should be "data ready" at launch. That means:

- It logs user activity with proper timestamps
- It allows linkage to CMS identifiers (e.g., BICN, MBI) under consent

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- It supports a minimum set of standardized impact tags (e.g., “appointment scheduled,” “medication refill requested”)

We’ve built these patterns into our platforms using FHIR, OMOP, and CDM mappings. CMS could require new vendors to follow similar structures to streamline evaluation from day one.

How SpinSys-Diné Can Support CMS

CMS has an opportunity to lead the market by embedding impact measurement as a core expectation for all digital health products aligned with care and payment flows, not bolted on after the fact.

SpinSys-Diné can help CMS make this vision actionable by bringing forward proven technical capabilities and implementation patterns:

- **Integrated claims-driven measurement**: We have built analytics pipelines in DHA’s MIP platform that link digital interventions to claims-based outcomes, including cost and utilization metrics. We can help CMS design similar data pathways that leverage linked claims and encounter data as the backbone for evaluating digital health impact.
- **Structured metadata collection**: We have operationalized DeviceUsage and Observation-based logging for VA and DoD health applications. We can help CMS define and implement lightweight, privacy-preserving usage telemetry standards that approved tools can adopt at launch.
- **Integration of wearable metadata**: SpinSys-Diné supported the integration of wearable metadata into contact tracing and situational awareness tools for the Defense Health Agency. During the early COVID response, we enabled the ingestion and analysis of passive health signals from wearables to identify patterns and exposures among service members. That experience demonstrated how structured, privacy-aware use of consumer health data can augment traditional clinical and claims data for public health decision-making. We can help CMS incorporate similar metadata streams from wearables into its real-world impact measurement framework—ensuring visibility into day-to-day health outcomes, not just clinical encounters or digital clicks.
- **Feedback loops from providers and plans**: SpinSys-Diné has built post-use feedback mechanisms (NPS, workflow impact) into Air Force and IHS clinical platforms. We can help CMS integrate similar structured feedback into provider workflows to capture context around digital tool use.
- **Academic and community partnerships**: We have supported mixed-method evaluation models in IHS and DoD environments, partnering with tribal and community organizations to evaluate technology adoption. We can help CMS engage similar networks to complement quantitative outcomes with qualitative insights.
- **Data ready by design**: We have embedded OMOP, FHIR, and CDM mappings into our platforms to ensure data readiness at launch, not retrofitted post-deployment. We can help CMS define minimum impact tagging requirements and reference implementations so that new tools meet "data ready" expectations from day one.

Across all of these areas, SpinSys-Diné can help CMS move impact measurement upstream, embedding it as a core design requirement for trusted digital health products that deliver measurable value to beneficiaries, caregivers, providers, and plans.

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PC-9 Given Blue Button 2.0 includes limited data, what additional CMS data would be most valuable to developers?

Blue Button 2.0 gave developers a starting point for working with CMS data, but its current scope is too limited to support high-impact applications. The data is mostly claims-based, lacks context, and arrives with a significant delay. To build meaningful tools for beneficiaries, caregivers, and providers, developers need more timely, detailed, and connected data delivered through consistent, well-documented APIs. CMS can unlock innovation by treating developers not just as users of open data, but as partners in public health. That means giving them what they need to build useful, safe, and scalable products.

Priority CMS Data Developers Need:

1. Real-Time or Near Real-Time Claims Status
Right now, developers only get finalized claims, often months after the encounter. Developers need:

- Pending claims information (even if preliminary)
- Status flags (e.g., “under review,” “denied with reason code”)
- Timing estimates for payment processing

This enables tools that help beneficiaries track out-of-pocket costs, understand denials, and manage medical debt in real time.

2. Coverage Details and Plan Rules
Developers need access to:

- Current plan enrollment status and coverage period
- Benefit structure: copays, deductibles, prior authorization requirements
- Plan-specific formularies and provider networks

We’ve seen in other federal programs (like TRICARE and VA Community Care Network (CCN)) that tools improve when benefit logic is transparent and machine-readable. CMS could offer a consistent plan rules API for Medicare Advantage, Part D, and Medigap.

3. Care Plan and Health Condition Flags
The claims feed shows what happened, not why. Developers need:

- Active diagnosis/problem list as known to CMS
- Chronic condition flags used for risk adjustment
- Preventive service gaps (e.g., overdue mammogram, no flu shot this season)

This data would allow tools to personalize guidance and improve navigation.

4. Prescription Drug Events (PDE) With Timeline Context
Part D data is essential, but developers need it with:

- Prescription start/end dates
- Days’ supply and refill tracking
- Prescriber and pharmacy information

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This would enable better medication adherence tools, refill reminders, and fraud prevention logic.

5. Provider Attribution and Encounter Context
Many beneficiaries have multiple providers. Developers need to know:

- Who CMS believes is the beneficiary's primary care provider
- Which providers are actively billing for care
- Encounter metadata: type of visit, reason code, site of service

This information enables tools to build out care teams, coordinate care plans, and align communications.

6. Data Provenance and Certainty Levels
When CMS shares derived data (e.g., HCC risk scores, quality metrics), developers need to know:

- Source system
- Date calculated
- Confidence level or revision history

SpinSys-Diné implemented similar data tagging in DHA systems and knows it's critical for developers working in high-trust domains like medication guidance or chronic care management.

How CMS Could Deliver This:

We recommend CMS invest in a **FHIR-based, modular API suite** that:

- Allows data to be scoped by user role (beneficiary, caregiver, developer)
- Provides clear documentation and versioning support
- Aligns with the HL7 CARIN and Da Vinci implementation guides
- Includes sandbox environments and sample payloads for testing

CMS should also expand the **developer support infrastructure** around Blue Button:

- Offer changelogs, bug reporting, and code samples
- Provide reference front ends to reduce duplicative work
- Create forums or advisory groups for feedback loops with developers

We've supported similar developer environments for DoD and IHS data services, including full-lifecycle API governance. We could assist CMS in building a scalable developer platform that accelerates adoption without compromising privacy or trust.

How SpinSys-Diné Can Support CMS

CMS has a critical opportunity to transform Blue Button from a limited data feed into a developer-ready, high-value data platform that drives innovation across the Medicare ecosystem. SpinSys-Diné can help CMS make this vision real by bringing proven capabilities and implementation experience:

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- **FHIR-based modular API design**: We have designed and implemented modular, versioned FHIR APIs for DHA and IHS data services, with flexible role-based access models and support for CARIN and Da Vinci implementation guides. We can help CMS define and deliver a scalable API suite that supports the evolving needs of beneficiaries, caregivers, and developers.
- **Data tagging and provenance models**: SpinSys-Diné has operationalized data provenance tagging, source lineage, and confidence scoring in DHA systems. We can help CMS implement similar models to ensure developers understand the source, quality, and trust level of CMS-derived data.
- **Developer sandbox environments**: We have built full-lifecycle developer environments for DoD and IHS APIs including sandbox instances, sample payloads, changelogs, and feedback loops. We can help CMS expand its developer platform with these proven patterns to foster safe and rapid adoption.
- **Cross-program data unification**: Our experience linking plan rules, provider directories, care gaps, and claims data in TRICARE and VA CCN programs positions us to help CMS build richer, more connected data sets that power next-generation beneficiary tools.
- **Privacy-first API governance**: We have supported API governance frameworks that balance developer innovation with strict privacy and security requirements aligning with federal standards. We can help CMS embed these governance patterns into its Blue Button evolution.

Across all of these efforts, SpinSys-Diné can help CMS move from a static data export model to an interactive, trusted data platform that enables better tools for beneficiaries, caregivers, and providers, while preserving CMS's role as a trusted steward of health data.

PC-12 What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

Operational health data is often the difference between a patient getting care on time or getting stuck in a loop of phone calls, confusion, and delays. For Medicare beneficiaries and caregivers, better access to operational data means fewer dropped handoffs, fewer missed services, and less frustration. It also introduces competition where patients can make informed choices about cost, quality, and convenience. CMS is well-positioned to make operational data more accessible, not by creating new tools, but by setting standards and expectations that make this data available to the tools patients and caregivers already use.

a. Examples may include the following:
(1) Binding cost estimates for pre-defined periods.

Patients need to know what they will owe not just per service, but for an entire episode such as a joint replacement or cataract surgery. Tools should show total expected out-of-pocket costs, what Medicare covers, and how different providers compare.

(2) Viewing provider schedule availability.

Patients should be able to view available appointment slots for primary, specialist, and diagnostic services. Without this, beneficiaries can't make informed choices even when better options exist.

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(3) Using third-party applications for appointment management.

CMS should enable certified applications to book, cancel, or reschedule appointments through provider APIs. This is especially valuable for caregivers managing complex care needs.

(4) Accessing patient-facing quality metrics.

Beneficiaries need quality metrics that reflect outcomes, patient experience, and accessibility not just process scores. Metrics should be understandable, relevant, and searchable by condition and location.

(5) Finding the right provider for specific healthcare needs.

Users should be able to search for providers based on condition, treatment goals, language spoken, or cultural preferences, not just ZIP code or specialty.

b. What use cases are possible today?

Some of these use cases are technically possible today, but access is inconsistent:

- **Cost estimates**: CMS collects pricing and cost-sharing data, but it is fragmented and not easily combined into consumer-facing tools.
- **Appointment scheduling**: Some providers expose scheduling APIs, but most use proprietary systems with no interoperability.
- **Quality metrics**: CMS publishes provider data through Care Compare, but it is not integrated into consumer applications or decision tools.
- **Provider search**: Medicare.gov allows basic lookup, but filters are limited and not tailored to patient-specific needs.

c. What should be possible in the near future?

Within the next 2–3 years, CMS could enable:

- **Standardized APIs for schedule availability**: Modeled on HL7 FHIR Slot and Schedule resources.
- **Bundled cost estimates**: Based on historical claims and price transparency rules, made available through APIs.
- **Open quality score service**: Where developers can pull relevant scores by provider, specialty, and geographic filters.
- **Interoperable provider directories**: Using validated NPPES and MA plan directories, exposed in machine-readable formats.

SpinSys-Diné could help CMS build reference implementations and compliance frameworks for these services, based on our experience linking operational data across DHA and VA systems.

d. What would be very valuable but may be very hard to achieve?

- **Real-time, personalized out-of-pocket cost projections**: Scaling this requires complete visibility into beneficiary plan design, accurate fee schedules, and up-to-date benefit usage data.

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We have delivered cost estimation engines at smaller scales, but scaling nationally requires harmonization across MA, Part D, and supplemental plans.

- **Full interoperability for appointment scheduling**: Achieving this across hundreds of EHR vendors with proprietary logic requires CMS to enforce standards and incentivize participation through payment or accreditation programs.
- **Outcome-based provider matching**: Building a smart referral engine that matches users to providers based on real-world outcomes would require deep EHR data integration, advanced risk adjustment, and trust-building with providers.

How SpinSys-Diné Can Support CMS

SpinSys-Diné can assist CMS in developing, testing, and scaling operational health data services by drawing on our experience delivering scalable, trusted platforms and user-centered tools across complex federal health environments:

- **Digital Health Navigation Capabilities**: Leveraging experience from MIDAS, LDCS, and CarePoint implementations, we unify coverage, care tasks, benefits, and health actions into integrated views—bridging fragmented systems to support whole-person care.
- **Family and Caregiver Coordination Features**: In deployments across MHS and IHS, we've enabled secure, multi-user access models with caregiver delegation, consent management, and coordinated navigation for families managing shared care responsibilities.
- **Conversational Guidance Functions**: Through rule-based logic and LLM integrations in VA and DHA systems, we provide plain-language explanations of coverage changes, benefit options, and care navigation steps—improving digital literacy and engagement.
- **Interoperability Standards Implementation**: Using frameworks proven in MIP and related platforms, we've delivered scalable data integration across FHIR, HL7, and OMOP—ensuring CMS can build on open, adaptable infrastructure.

CMS does not need to solve all these challenges alone, but it can lead by setting standards, incentivizing participation, and ensuring that every Medicare beneficiary, regardless of ZIP code or digital skills, can benefit from better access to operational health data. SpinSys-Diné is ready to support this effort with proven tools and experience integrating operational data across some of the most complex federal health environments.

PR-2 What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

Useful, innovative applications for physician workflows already exist. But most don't reach clinical practice or fail to deliver impact once deployed. The problem isn't always technology. The problem is fragmentation, misaligned incentives, and friction inside the clinical environment. The result is burnout, low trust in digital tools, and lost opportunities to improve quality, reduce cost, and improve care delivery.

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CMS is uniquely positioned to reduce this friction not by building software, but by clearing the policy, data, and workflow barriers that block innovation at the point of care. Physician workflows are often trapped in systems that lack scalable AI integration, depend on outdated interfaces, and rely on fragmented, inconsistent data. This leads to manual documentation, delayed reporting, and lost efficiency. SpinSys-Diné can help CMS address these barriers by deploying proven tools that simplify integration, automate routine work, and enable scalable innovation.

How SpinSys-Diné Can Support CMS

CMS can accelerate the development and deployment of innovative physician workflow tools by removing data, policy, and workflow barriers. SpinSys-Diné can help CMS operationalize this vision with proven capabilities deployed in some of the most complex federal health environments:

- **GENOME**: Our AI-powered code transformation engine automatically modernizes legacy systems enabling EHR-agnostic integration of new tools and APIs within existing clinical environments.
- **Big Data Virtualization (BDV)**: We unify clinical, billing, and operational data into a single access layer that supports real-time quality dashboards, predictive workflows, and compliance logic. This reduces data fragmentation and powers advanced clinical decision support.
- **Natural Language Processing (NLP) for documentation**: We have deployed NLP solutions that extract structured data from clinical notes, reducing documentation burden while improving data quality and billing accuracy directly aligned with CMS's goal to reduce clinician burnout.
- **Quality automation and API-based billing**: SpinSys-Diné has implemented measure logic abstraction, workflow-triggered reporting, and open billing APIs in DHA systems creating scalable, flexible quality measurement that adapts to CMS's evolving program needs.

In addition, we can help CMS scale key enablers across the ecosystem:

- **FHIR-based interoperability frameworks**: We have delivered reusable FHIR connectors and validation tools for DHA and IHS, which can help CMS enforce consistent, standards-based API adoption across vendors.
- **Burden-reducing incentive alignment**: We have built and deployed workflow simplification tools for military and tribal health systems and can help CMS develop program models (APMs, MIPS) that reward similar improvements in civilian care.
- **Open API governance**: SpinSys-Diné has implemented EHR-agnostic integration layers and open API governance frameworks in DoD and IHS systems. We can help CMS structure its programs to promote open ecosystems and prevent vendor lock-in.
- **Digital-first quality measure libraries**: We have translated complex quality measures into CQL and developer-ready libraries, enabling rapid adoption by tool developers and alignment with CMS's future digital-first measure strategy.
- **Clinician-centered testing frameworks**: We have supported clinician-led user acceptance testing (UAT) processes at scale in large federal health systems. We can

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help CMS fund and institutionalize clinical-user testing labs to ensure new tools fit real-world workflows.

SpinSys-Diné is ready to help CMS design and operate the frameworks, reference implementations, and testing environments needed to unlock next-generation physician workflow innovation—reducing clinician burden, improving data quality, and enabling more effective value-based care.

PR-3 How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format?

It is essential that all data in an Electronic Health Record (EHR) system be accessible for exchange, regardless of storage format, if CMS wants to improve care delivery and coordination in both urban and rural settings. Without full access to structured and unstructured data, clinicians operate with partial information. That leads to missed diagnoses, duplicative services, unsafe prescribing, and avoidable readmissions.

This is not theoretical. We've seen its impact in tribal clinics, rural hospitals, and large integrated health systems. Whether in a small community clinic or a major metro network, delays in accessing the full patient record lead to fragmented care and higher costs.

Why This Matters in Urban and Rural Areas

In **rural areas**, patients move between local clinics and distant hospitals for specialty or acute care. Often, they return with no usable electronic record. Faxed documents and mailed summaries arrive incomplete or too late.

In **urban systems**, patients generate massive volumes of data across providers and settings, but systems don't share well. With multiple specialists and frequent transitions, clinicians work with blind spots, relying on patient recall or making unsafe assumptions.

In both contexts, a lack of full EHR access creates safety risks, inefficiencies, and higher administrative burden.

Why Storage Format Should Not Be a Barrier

EHR vendors often store data in proprietary formats, split structured and unstructured fields, or lock content inside attachments. This blocks third-party tools, referral systems, and public health programs from accessing:

- Scanned forms, faxes, and handwritten notes
- Narrative radiology or pathology reports
- Social determinants assessments
- Advance directives or care plans stored as PDFs
- Consent records and correspondence

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When this data remains locked, “interoperability” becomes limited to medication lists and problem codes. That’s not enough to coordinate real care.

How SpinSys-Diné Can Support CMS

SpinSys-Diné can help CMS make full EHR access a reality across all care settings, both urban and rural, by bringing proven technical capabilities and operational experience:

- **Big Data Virtualization (BDV)**: We unify structured and unstructured clinical data including scanned documents, PDFs, narrative reports, social determinants assessments, and care plans into a single interoperable layer. This enables full longitudinal record exchange via FHIR, CDA, and bulk data formats.
- **GENOME**: Our AI-powered code transformation engine extracts, normalizes, and modernizes legacy EHR content making previously locked data accessible through standard APIs and usable in real-time clinical workflows.
- **AI/ML-driven data fidelity**: We apply machine learning to fill data gaps, improve accuracy across modalities, and enhance data quality for CMS quality reporting and value-based programs.
- **Provenance and traceability**: Our solutions enforce robust data provenance and traceability, giving clinicians confidence in the origin, quality, and completeness of exchanged data.
- **Field-proven scalability**: We have used these tools to create interoperable layers between legacy EHRs and modern cloud platforms in federal systems, including large-scale urban networks and rural sites with older infrastructure.

With these capabilities, SpinSys-Diné can help CMS define technical guidance, develop reference patterns, and implement validation frameworks to ensure that full-record access across all storage formats is feasible, secure, and scalable.

What CMS Could Do

CMS can lead by requiring that all certified EHRs expose clinically relevant data, including structured and unstructured content, through open, validated interfaces. This should include:

- Clinical notes and narrative summaries
- Patient-reported outcomes and SDOH inputs
- Scanned forms, diagnostic narratives, and consults
- Referral loops and care plan attachments

CMS could embed these requirements into:

- TEFCA participation standards
- Conditions of Participation
- ACO REACH, MSSP, and Medicaid transformation initiatives

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SpinSys-Diné can support CMS with technical guidance, reference patterns, and validation testbeds to ensure that full-record access is delivered consistently across the ecosystem.

If CMS wants a digital health system that works for everyone, it cannot allow clinically relevant data to remain siloed by format. Access to the full record regardless of storage method is essential for safe, equitable, and coordinated care. SpinSys-Diné is ready to help CMS build the infrastructure to make that a reality.

PR-7 What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability?

CMS can support providers in delivering high-quality, timely, and comprehensive data for interoperability by shifting from mandates to enablement. Providers want to share data but many lack the time, tools, or staffing to do it consistently or securely. The biggest gaps aren't technical. They're operational. CMS can close those gaps with a mix of clear expectations, shared infrastructure, validation support, and actionable feedback. Here's how:

1. Fund Shared Data Infrastructure for Low-Resource Providers

Small and rural practices often lack dedicated IT resources. They rely on vendors or regional networks to manage interoperability.

CMS could:

- Fund regional data hubs offering FHIR APIs, transformation services, and reporting infrastructure
- Provide opt-in "shared gateways" for Medicaid and Medicare exchange
- Offer direct grants through HITECH-style mechanisms

We've delivered shared data infrastructure across tribal and IHS networks. These models work when coupled with operational and financial support.

2. Make Data Quality Visible and Actionable

Providers often receive no feedback on the quality of submitted data.

CMS should:

- Deliver real-time dashboards that flag data gaps, delays, and formatting errors
- Align alerts with EHR workflows (not email inboxes)
- Show how data quality affects reimbursement, quality metrics, and care delivery

In our DHA and MDS programs, we've operationalized continuous data quality scoring and embedded feedback directly into clinical tools. These loops help providers catch and fix problems before data moves downstream.

3. Offer Test Beds and Validation Tools

Even willing providers can't test whether their data is compliant.

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CMS should offer:

- Test sandboxes with synthetic patient scenarios
- Self-service validation tools for FHIR payloads, terminologies, and logic
- Open-source mappings for common clinical and quality measures

We've stood up national-scale test environments and helped providers use them in real-world implementations. We could help CMS scale these as a national resource.

4. Reduce Redundant Reporting Burdens

Many providers re-enter the same data for different programs: CMS, Medicaid, plans, and registries.

CMS could:

- Harmonize reporting logic across federal and state programs
- Offer centralized APIs that distribute data across endpoints
- Auto-fill forms using previously submitted, structured data

We've done this in integrated tribal/DoD settings, where removing duplicate submission pathways earned immediate provider support.

5. Incentivize "Data Readiness" in Payment Models

CMS could reward providers that meet data benchmarks such as:

- Near real-time clinical data submission
- Full participation in FHIR-based exchange
- Consistent use of standard vocabularies (SNOMED, RxNorm, LOINC)

In our programs, we've added AI-driven anomaly detection to flag potential data errors and surface them before submission. These pipelines reduce noise and improve signal quality without adding manual work.

6. Clarify Legal and Operational Expectations

Many providers hesitate to share data because of unclear policies.

CMS could:

- Publish "data sharing playbooks" tailored by provider type and care setting
- Provide templated legal agreements (e.g., consent, use authorizations)
- Offer scenario-based guidance for youth, SUD, disaster, and cross-jurisdictional cases

We've supported data governance for HHS and DoD systems using Policy Manager + BDV integration ensuring compliance is enforced in process, not just policy.

7. Promote Local Champions and Peer Support

Behavior change requires trusted messengers.

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CMS could:

- Fund peer-to-peer training programs
- Train and empower super-users within networks
- Share success stories and playbooks across regions and provider types

We've seen this model work through QIOs and modernization projects. It scales when providers see their peers succeed.

How SpinSys-Diné Can Support CMS

CMS can help providers deliver high-quality, timely, and comprehensive data by embedding enablement-first strategies across its programs. SpinSys-Diné can help CMS implement these strategies with proven capabilities and field-tested solutions:

- **AI-supported data quality pipelines**: We have operationalized continuous data quality scoring and anomaly detection in DHA and MDS platforms enabling near real-time feedback to providers and improving submission fidelity without adding manual work.
- **Operational testbeds and validation tools**: SpinSys-Diné has built national-scale test environments for DoD and IHS systems, including FHIR payload validation, synthetic patient scenarios, and self-service tools. We can help CMS deploy similar testbeds as a national interoperability resource.
- **Shared data infrastructure**: We have delivered regional and shared data hubs for tribal and IHS networks, supporting FHIR APIs, transformation services, and multi-program data exchange. We can help CMS expand shared infrastructure models for low-resource providers.
- **Governance frameworks and policy automation**: In HHS and DoD environments, we have implemented Policy Manager + BDV integrations that enforce data sharing rules in-process, not just in documentation. We can help CMS deliver governance frameworks and templated legal agreements that promote trusted, compliant data exchange.
- **Workflow simplification and duplicate reduction**: We have harmonized reporting logic and automated data reuse across DoD and Tribal programs, reducing redundant reporting burdens and increasing provider participation in interoperability initiatives.
- **Clinician-centered enablement**: We have supported peer-led enablement models and QIO-driven modernization projects that drive adoption through trusted local champions. We can help CMS scale similar programs to promote data readiness and sharing best practices across the provider community.

SpinSys-Diné is ready to help CMS build the tools, infrastructure, and governance patterns that make high-fidelity interoperability the default, not the exception, across all Medicare and Medicaid programs.

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PR-8 What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

CMS can help simplify providers' clinical quality data responsibilities by reducing duplicative work, providing tools that fit real workflows, and making the reporting logic clearer and more consistent. Right now, many providers see quality reporting as an administrative burden rather than a care improvement tool. That's because reporting is often disconnected from day-to-day clinical work.

We've seen this challenge across DoD, VA, IHS, and tribal health environments. The core problems are complexity, fragmentation, and poor feedback. CMS can fix this with better alignment, automation, and communication.

What CMS or Partners Can Do

1. Pre-Fill and Automate Where Possible

- Auto-populated templates inside EHRs tied to specific measures
- Default logic libraries that extract quality data from existing charting
- Standardized value sets across programs to eliminate code mapping

In our federal health work, SpinSys-Diné has automated the extraction and transformation of clinical quality data using **Bulk FHIR and Big Data Virtualization (BDV)**. These solutions:

- Reduce reporting cycles from weeks to hours
- Eliminate duplicative data entry through end-to-end data flow automation
- Support real-time quality monitoring by integrating registry feeds with BI and AI dashboards

CMS could adopt these models to shift quality calculations to trusted BDV-based services, while giving providers live visibility into their performance.

We've also delivered automated abstraction engines that reduced reporting time by over 60% in government clinical systems. We could help CMS scale these solutions across ambulatory and small-practice settings.

2. Simplify and Align Measures Across Programs

CMS should eliminate redundant measures and consolidate requirements across:

- MIPS
- ACO models
- Medicare Advantage
- Medicaid managed care

A single core measure set, with optional extensions, would reduce confusion and avoid duplicated submissions. CMS could also release an annual "measure change bulletin" that clearly outlines what's new, retired, or updated.

3. Offer Self-Service Tools for Validation and Submission

Many providers don't know if they've submitted the right data or why their performance dropped.

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CMS should offer:

- Measure-specific validation tools with sample payloads and logic walk-throughs
- Submission sandboxes where providers can test files before official reporting
- Web-based dashboards to show completeness, errors, and trends over time

We've built similar self-service interfaces for federal claims and quality data systems and could help CMS develop an integrated platform for provider use.

4. Allow Delegation to Trusted Third Parties
Small practices often don't have the time or staff to manage reporting.

CMS should support:

- Approved third-party data submitters or registries
- API-based delegation for data pulls from EHRs or clearinghouses
- Shared service arrangements across health centers or ACOs

We've supported delegated data collection in tribal and public health systems and could advise CMS on role-based access, audit trails, and provider control.

5. Provide Feedback That Drives Action
Providers are more likely to invest in quality reporting when they see clear results.

CMS should offer:

- Peer comparisons based on similar patient panels
- Breakdown of performance by care team, site, or time period
- Narrative feedback with suggested actions to improve

This mirrors what we've seen work in clinical quality improvement collaboratives, where tailored feedback drives faster behavior change than generic scores.

6. Develop Embedded Education in Clinical Tools
Instead of separate trainings, CMS could embed quick tips and definitions inside tools. For example:

- Hover-over definitions for each quality measure
- Examples of good documentation in the EHR
- Prompts during charting tied to quality opportunities

We've supported user-centered design for provider tools that pair education with action and could help CMS make this part of standard quality tooling.

How SpinSys-Diné Can Support CMS

CMS can simplify clinical quality data work by making it automatic where possible, aligned across programs, and easy to understand. SpinSys-Diné can help CMS and its partners deliver this vision with proven tools and field-tested implementation experience:

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- **Bulk FHIR and BDV-powered automation**: We have automated end-to-end extraction, transformation, and reporting of clinical quality data using Bulk FHIR and Big Data Virtualization (BDV) pipelines reducing reporting cycles from weeks to hours and eliminating duplicate data entry.
- **Automated abstraction engines**: SpinSys-Diné has deployed automated abstraction engines in federal clinical systems reducing manual reporting time by over 60% and enabling real-time performance monitoring.
- **Self-service validation and submission tools**: We have built measure-specific validation tools, test sandboxes, and self-service dashboards for federal claims and quality data systems. We can help CMS deliver similar tools to providers to improve data accuracy and trust.
- **Delegated data collection frameworks**: SpinSys-Diné has implemented delegated data collection models with role-based access, audit trails, and provider control in tribal and public health systems. We can help CMS enable secure, trusted third-party submission pathways.
- **Embedded education and decision support**: We have supported user-centered design for provider tools that embed education and action prompts directly into clinical workflows. We can help CMS integrate similar capabilities into quality reporting tooling to drive behavior change at the point of care.
- **Program alignment and measure harmonization**: Our experience harmonizing reporting logic across DoD, Tribal, and IHS programs positions us to help CMS streamline quality measure definitions and reduce redundant reporting burdens across federal and state programs.

SpinSys-Diné is ready to help CMS design and deploy the next generation of clinical quality data tools that support providers at the point of care, not after the fact, and that enable quality reporting to serve its true purpose: improving patient outcomes.

TD-1 What short term and longer-term steps can CMS take to stimulate developer interest in building digital health products?

To stimulate developer interest, CMS needs to reduce friction, show there's a viable path to adoption, and build trust in the value of participating. Developers don't just want access to data. They want clarity, consistency, and a fair shot at impact. CMS can deliver that by treating developers as part of the public health infrastructure not just software vendors.

Short-Term Steps (Next 6–12 Months)

1. Create a Single, Well-Documented API Gateway

Developers need one place to find and use CMS data and services. CMS should:

- Launch a unified developer portal with sandbox access, live documentation, and sample payloads
- Combine Blue Button, Medicare Coverage APIs, Quality APIs, and other tools into one gateway
- Include FHIR profiles, token request flows, and consent logic examples

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We've supported unified data access in federal programs, and this step alone removes weeks of startup friction.

2. Publish Clear Use Cases and Success Paths

Too many developers don't know what CMS wants built. CMS should:

- List specific challenges that digital tools could solve (e.g., digital navigation for dual-eligibles, chronic condition management in rural areas)
- Showcase approved use cases and real integrations from existing vendors
- Make clear what success looks like: user adoption, clinical outcomes, claims impact

This gives developers a target and increases confidence in market fit.

3. Offer Microgrants or Challenge Prizes

Small, focused grants tied to measurable prototypes (e.g., "build a working Medicare appointment finder with FHIR scheduling") help early-stage teams engage without waiting for a full procurement cycle.

CMS could use a challenge platform, work through ONC, or partner with health tech incubators.

4. Open Real Test Data and Validate Environments

Without test data, development stalls. CMS should provide:

- Fully synthetic FHIR patient data sets with edge cases and longitudinal events
- Pre-configured dev environments (e.g., Docker containers or Postman collections)
- Validation endpoints to test payloads before going live

We've delivered testbeds for other federal data programs. Developers use them to build quickly and correctly.

Longer-Term Steps (1–3 Years)

1. Certify and List CMS-Approved Tools for Care Management and Patient Use

Developers want a clear path to adoption. CMS should:

- Create a registry of tools that meet CMS data, security, and usability standards
- Allow providers, payers, and ACOs to search and adopt from this list
- Connect registry inclusion to participation in APMs, MA plans, or value-based initiatives

We've supported similar pathways in DoD and IHS systems that helped smaller vendors scale adoption.

2. Allow Reuse of Tools Across CMS Programs

Right now, a developer building for Medicare FFS can't easily reuse the same tool for Medicaid, CHIP, or Medicare Advantage. CMS should standardize:

- Consent flows
- Data payloads

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- Reporting requirements

This reduces rebuilds and helps products scale across the CMS ecosystem.

3. Fund Technical Assistance and Community Building

CMS could support a national digital health developer program that includes:

- Office hours and tech support from CMS teams
- Developer forums and working groups
- Regional events co-hosted with states or provider groups

This turns one-way documentation into an active, learning ecosystem.

4. Create Reimbursable Codes for Digital Interventions

To attract long-term investment, CMS should allow certain digital health interventions, such as chronic care navigation or digital SDOH screening to be reimbursable under APMs or fee-for-service pilots. This creates a business model for developer-backed delivery.

How SpinSys-Diné Can Support CMS

CMS can accelerate developer engagement by delivering unified APIs, actionable use cases, trusted test environments, and scalable adoption pathways. SpinSys-Diné can help CMS design and implement this developer ecosystem with proven capabilities and federal health experience:

- **Unified developer portals and API gateways**: We have built unified data access platforms for federal health programs combining multiple APIs, live documentation, and validation tools into a single, developer-friendly gateway.
- **Synthetic data testbeds**: Our Synthetic Data Engine (SDE) generates fully synthetic patient data sets, including edge cases and longitudinal events, for safe and realistic testing. We have deployed these testbeds for federal data programs to accelerate developer onboarding and innovation.
- **Developer enablement infrastructure**: We have supported national-scale developer communities with pre-configured dev environments, validation endpoints, and self-service tools enabling faster, more consistent adoption of APIs and data services.
- **Certification and registry frameworks**: SpinSys-Diné has helped federal agencies define certification pathways and registries for digital health tools connecting developers to real adoption channels through APMs, MA plans, and value-based initiatives.
- **Cross-program interoperability**: We have implemented standardized consent flows, data payloads, and reporting logic across DoD, and IHS systems reducing rebuilds and supporting scalable product deployment across multiple federal programs.

SpinSys-Diné is ready to help CMS design a vibrant, trusted developer ecosystem that treats developers as partners in public health infrastructure driving scalable innovation that improves patient outcomes and system performance.

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TD-5 How could a nationwide provider directory of FHIR endpoints improve access to health information?

A nationwide provider directory of FHIR endpoints would improve access to health information by making automated, reliable, and secure exchange possible at scale. Today, too many systems can technically exchange data but don't know where to send it, who can receive it, or how to format it. This leads to delays, faxes, duplicate tests, and missed care transitions.

CMS can fix that. A verified, publicly accessible directory of FHIR endpoints would allow payers, providers, and digital health tools to locate and connect with the right endpoint at the right time with no guesswork.

Why It Matters

1. Point-of-Care Access for Transitions and Referrals

When a patient is discharged or referred, care teams often don't know where to send clinical summaries or care plans. A national FHIR endpoint directory would:

- Show which provider organizations accept FHIR-based messages
- Identify the correct endpoint based on NPI, TIN, or facility ID
- Include metadata like endpoint use case (e.g., clinical summary, quality data, scheduling)

We've supported transitions of care systems for IHS and VA that failed when receiving endpoint info was missing or outdated.

2. Streamlined Patient Data Sharing

Patients moving across systems, for example from an MA plan to FFS or from hospital to home care, often find their data trapped. With a provider endpoint directory:

- Digital tools could auto-query where a provider stores FHIR data
- Applications could request or push data using SMART-on-FHIR workflows
- Consent management systems could route records securely

This removes friction from patient-directed exchange and third-party app access.

3. Accelerated Public Health and Quality Reporting

CMS, state agencies, and registries often request data from thousands of providers. Without a directory, outreach requires custom configurations or fax fallback.

With a verified directory:

- CMS could automate record pulls for quality measurement, risk scoring, or research
- ACOs and plans could streamline chart retrieval for shared patients
- State public health systems could integrate real-time surveillance

We've implemented automated pull mechanisms for quality systems like MIDAS and IQR, where missing endpoint data often breaks the pipeline.

4. Support for Provider Matching and Attribution

Many CMS models depend on correctly assigning patients to providers. Endpoint directories help by:

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- Validating provider identity and location
- Supporting accurate provider search and network composition
- Enabling real-time record requests during enrollment or care onboarding

We've worked with MA and Medicaid plans trying to match NPIs to active systems. A unified FHIR directory would simplify that across markets.

What CMS Could Do

To make this real, CMS could:

- Fund and operate a national, queryable FHIR endpoint directory tied to NPPES and PECOS
- Require MA plans, ACOs, and certified EHR vendors to register active endpoints with standardized metadata
- Validate endpoints routinely (e.g., ping checks, status updates)
- Provide an open API for digital tools, payers, and public health systems to use the directory in real time

CMS could model this on existing TEFCA directory work or Da Vinci's Plan-Net profile but go further by linking it directly to CMS programs and incentives.

How SpinSys-Diné Can Support CMS

A nationwide provider FHIR endpoint directory would transform potential interoperability into real-world, scalable data exchange supporting more accurate transitions, public health reporting, patient-directed exchange, and quality measurement. SpinSys-Diné can help CMS design, validate, and maintain this directory with proven tools and field-tested patterns:

- **Automated FHIR endpoint validation**: We have implemented automated endpoint validation mechanisms (ping checks, status updates) for quality systems such as MIDAS and IQR ensuring that directory entries remain accurate and actionable.
- **FHIR directory integration with provider identity and attribution**: SpinSys-Diné has supported NPI-to-system mapping and provider matching for MA and Medicaid plans enabling accurate patient-provider attribution and trusted data exchange.
- **Consent-aware routing frameworks**: We have built consent management and routing frameworks that leverage directory metadata enabling secure, patient-directed exchange and third-party app integration.
- **Real-time pull mechanisms**: SpinSys-Diné has implemented automated record retrieval pipelines for quality measurement and public health surveillance using FHIR directory data to drive real-time, reliable data flows.
- **FHIR directory governance and architecture**: We have helped federal agencies design and maintain scalable, queryable directory services integrating with NPPES, PECOS, and national directory standards (Da Vinci Plan-Net, TEFCA directory profiles).

SpinSys-Diné is ready to help CMS deliver a trusted, validated nationwide FHIR endpoint directory that enables real-time, scalable health information exchange—turning "we could connect" into "we did connect" for patients, providers, and public health partners.

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VB-1 What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Accountable Care Organizations (ACOs) and Medicare Shared Savings Program (MSSP) participants already have a financial interest in improving care coordination and outcomes; however, digital health management and navigation tools are still underused. The reason is simple: most APMs don't see a clear return on investment, and many digital tools don't align with their workflows or data infrastructure. CMS can shift that by making these tools more valuable, easier to use, and directly tied to performance and payment.

Incentives CMS Could Implement

1. Shared Savings Credit for Verified Digital Use

CMS could offer bonus points or higher savings rates to ACOs that deploy digital tools that meet minimum use thresholds, for example:

- % of patients enrolled in a digital care management app
- % of high-risk patients using tools to manage chronic conditions
- % of transitions of care managed through a digital navigator

Use could be tracked through standardized FHIR logs or CMS-defined data tagging. This rewards behavior without prescribing a specific vendor.

2. Flexible Use of Shared Savings to Invest in Digital Tools

Many ACOs hesitate to buy tools because they can't bill for them or don't know if they'll get savings. CMS could allow a portion of shared savings to be pre-authorized for reinvestment into digital tools similar to how community benefit reinvestment works in Medicaid 1115 waivers. We've helped similar reinvestment models work in public health and tribal programs, where upfront infrastructure support improves long-term outcomes.

3. Upfront Per-Beneficiary Payments (PBPs) for High-Need Populations

CMS could offer targeted PBPs to ACOs for deploying digital navigation tools to specific populations such as dual-eligibles, high-cost chronic conditions, or recently discharged patients. These payments could be tied to digital engagement metrics (e.g., daily logins, medication adherence reminders sent). This structure de-risks adoption and focuses ACOs on populations where digital tools make the biggest impact.

4. Digital Tool Certification + Alignment with Quality Measures

CMS could publish a list of certified digital tools that directly support MIPS or MSSP quality measures (e.g., blood pressure control, A1C management). ACOs using these tools could receive measure credit or pre-filled abstraction logic. We've delivered tools that automate data capture and abstraction from digital front ends to structured data stores. We could help CMS align tool functionality with existing quality logic libraries.

5. Public Recognition and Market Differentiation

CMS could publish digital engagement performance at the ACO level similar to star ratings. This visibility encourages competition and shows that CMS values innovation.

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Benchmarks could include:

- Digital enrollment by risk tier
- Use of digital follow-up after hospital discharge
- Integration of patient-reported outcomes through digital tools

6. Support for Data Integration and Infrastructure

Even when ACOs want to use digital tools, data silos block implementation.

CMS could offer grants, technical assistance, or shared services to help ACOs:

- Link digital tools to EHRs, registries, and claims feeds
- Support FHIR API adoption and test validation
- Connect community-based and tribal providers to digital infrastructure

We've supported this integration work across VA, DHA, and IHS systems, often helping bridge EHR vendors with public cloud and government data streams. We could help CMS replicate that support model.

The most effective incentives are those that reduce risk, align with payment, and create visible value. CMS doesn't need to mandate digital health tools but it can make smart use of carrots: more savings, better quality scoring, and easier implementation support. We could help CMS design, test, and scale those programs by reusing real patterns from government health systems where this work is already underway.

To accelerate adoption, CMS could also tie financial incentives directly to digital patient engagement metrics such as care gap closures, timely follow-ups, or reduced avoidable ER visits. We've used this approach in the Military Health System by integrating dashboards that connected digital outreach to clinical outcomes, prompting real behavior change.

How SpinSys-Diné Can Support CMS

CMS can accelerate adoption of digital health management and care navigation tools in APMs by providing aligned incentives, data integration support, and operational infrastructure. SpinSys-Diné can help CMS implement these strategies with proven capabilities and real-world experience:

- **Automated data capture and abstraction**: We have delivered tools that automate data capture and abstraction from digital front ends into structured data stores, enabling alignment with MIPS and MSSP quality measures and reducing clinician burden.
- **Support for reinvestment models**: We have operationalized community benefit reinvestment models in public health and tribal programs, helping organizations channel upfront infrastructure investments into digital tools that drive long-term outcomes.
- **Digital engagement metrics integration**: In the Military Health System, we have integrated dashboards that link digital outreach to clinical outcomes (e.g., care gap closures, timely follow-ups, reduced avoidable ER visits). This is a model we can help CMS adapt for ACO performance tracking.
- **FHIR-based data integration and infrastructure**: SpinSys-Diné has supported FHIR API adoption, test validation, and multi-source data integration across VA, DHA, and IHS

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systems bridging EHR vendors with public cloud and government data streams. We can help CMS scale similar integration support for ACOs.

- **Certification and quality measure alignment**: We can assist CMS in defining digital tool certification criteria and aligning certified tool functionality with quality measure logic libraries drawing on our experience building automated abstraction engines and quality data pipelines.

SpinSys-Diné is ready to help CMS design, test, and scale digital incentive frameworks that reduce ACO adoption risk, align with payment models, and drive measurable improvements in patient engagement, care quality, and outcomes.

VB-2 How can AI, analytics, risk stratification, usability, and engagement be integrated into APMs?

CMS can integrate AI, analytics, risk stratification, usability, and engagement into Alternative Payment Models (APMs) by treating them not as optional add-ons, but as core infrastructure for delivering outcomes-based care. Right now, most ACOs and MSSP participants rely on static dashboards, retrospective claims data, and manual processes. That makes it hard to identify risk early, intervene at scale, or close care gaps efficiently.

To succeed under APMs, providers need more than data. They need tools that interpret that data, present it in usable ways, and drive timely, trusted actions. CMS can make that possible by embedding these capabilities into the architecture of APM operations, payment incentives, and quality expectations.

How Each Element Can Be Integrated

1. AI and Machine Learning

AI can help identify care gaps, predict risk, and prioritize outreach. But it must be transparent, explainable, and tied to reimbursement logic.

CMS could:

- Provide a list of approved AI use cases (e.g., avoidable readmission risk, missed preventive screenings)
- Define baseline performance metrics (e.g., sensitivity, false-positive rates) for ACOs using AI to drive interventions
- Encourage use of AI-driven insights through quality measure pre-filling or shared savings uplift

We’ve implemented AI models that triage patients for care coordination teams based on social risk, prior utilization, and digital engagement. We could help CMS validate and scale similar approaches.

2. Analytics for Operational Decision-Making

Most ACOs lack real-time insight into performance. CMS should push analytics into daily workflows.

CMS could:

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- Fund self-service dashboards that show rolling performance against cost and quality targets
- Require digital tools to expose embedded analytics for care managers, not just executives
- Offer optional APIs for pulling CMS risk and utilization data directly into ACO tools

We've delivered operational dashboards for federal clients with thousands of users, showing live gaps, trends, and predicted penalties. These help turn analytics into action.

3. Risk Stratification Tied to Resource Allocation

Risk scores alone don't drive better outcomes. What matters is whether teams use them to decide who gets what kind of care and when.

CMS could:

- Require ACOs to define risk-tiered care models (e.g., low-risk = self-management tools, high-risk = human outreach + digital care plan)
- Tie payment to documented action based on risk (e.g., "High-risk diabetic patient received nutrition consult + navigator call")
- Promote use of composite risk models that include clinical, social, and engagement indicators

We've developed such stratification engines for government care programs using SDOH, claims, and digital usage signals. We could help CMS build standard logic and templates.

4. Usability Standards for Tools in APMs

If tools are hard to use, they won't be used no matter how powerful. CMS should embed usability into APM participation.

CMS could:

- Publish baseline usability standards for digital tools (e.g., mobile responsive, 6th grade reading level, offline access)
- Tie care coordination payments to use of tools meeting those standards
- Collect user satisfaction feedback as part of APM evaluations
- Offer a CMS usability SDK for developers to streamline adoption of required design patterns and telemetry standards

We've supported UX validation for federal health applications targeting seniors, caregivers, and veterans. We've also built developer toolkits, including modular SDKs, that ensure consistent, standards-based interfaces, logging, and accessibility across platforms. SpinSys-Diné can help CMS define usability benchmarks and deliver a supporting SDK that simplifies compliance for developers and improves the user experience for patients and providers.

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5. Engagement as a Measurable Component

CMS often focuses on cost and clinical outcomes, but engagement, whether patients actually interact with their care plan, is what enables those results.

CMS could:

- Track digital engagement rates across ACOs (e.g., % of members who open care messages, complete digital check-ins, or update goals)
- Allow ACOs to earn credit for sustained engagement among hard-to-reach populations
- Use engagement patterns as part of quality or shared savings modifiers

We've built engagement metrics for DHA platforms using secure messaging, mobile surveys, and check-in completion. These helped flag drop-off and improve retention.

AI, analytics, risk scoring, usability, and engagement are not separate pillars. They are connected functions that make APMs more actionable, equitable, and scalable.

CMS can bring these into the model by:

- Requiring documentation of risk-based action
- Funding integrated analytics infrastructure
- Certifying usable tools

How SpinSys-Diné Can Support CMS

CMS can transform APMs by making AI, analytics, risk stratification, usability, and engagement core components of program operations, not optional enhancements. SpinSys-Diné can help CMS deliver this transformation with proven tools and real-world experience:

- **Synthetic Data Engine (SDE)**: We enable safe development and testing of AI models without PHI powering early detection, automated triage, and personalized risk targeting for high-need populations.
- **AI-based care coordination and alerting**: Using BDV and SDMA event services, we have deployed AI-driven care coordination tools that deliver actionable insights to care managers and automate outreach workflows aligning with APM performance targets.
- **Explainable analytics through MDACA**: Our Multiplatform Data Acquisition, Collection, and Analytics (MDACA) provides real-time, explainable risk scoring, performance dashboards, and transparency features, ensuring that predictive models are trusted and actionable at the point of care.
- **Personalized patient engagement**: We have built privacy-safe patient engagement platforms using SDE-powered tooling including mobile surveys, check-ins, and secure messaging improving retention and care plan adherence in DHA and CMS environments.

SpinSys-Diné is ready to help CMS bring these capabilities into standard APM operations and certification ensuring that AI, analytics, risk scoring, usability, and engagement become integral to outcomes-driven care models that improve patient experience, equity, and value.

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VB-3 What are essential IT capabilities for value-based care?

Value-based care (VBC) depends on timely, accurate information delivered to the right people, in the right format, at the right time. IT systems that support VBC must do more than store data. They must make data usable, align it with incentives, and help teams act on it without adding burden.

CMS can lead by defining essential IT capabilities as infrastructure, not as optional add ons, across ACOs, MSSP participants, and state partners. These capabilities support population health management, care coordination, cost control, and outcome tracking.

Essential IT Capabilities for Value-Based Care

1. Longitudinal Patient Data Access Across Settings
VBC requires a full picture of the patient journey not just what happened inside a single clinic. IT systems must:

- Aggregate data from EHRs, claims, labs, pharmacy, and social services
- Provide longitudinal views (e.g., problems, care plans, gaps)
- Support query-based and event-based access via FHIR or other APIs
- SpinSys-Diné's MDACA platform enables this real-time data integration across payer and provider systems, normalizing records and supporting longitudinal care planning. We've used this approach across DoD and tribal health environments.

2. Risk Stratification and Predictive Analytics
Care teams must know who is at greatest risk and what kind of support they need. Systems must include:

- Tools to generate risk tiers from claims, EHR, and social factors
- Predictive flags (e.g., likely readmission, ER overuse)

SpinSys-Diné has deployed AI-powered analytics at DHA and in CMS MIP environments to drive stratification and early intervention. These models also support attribution logic and AI-based patient ID matching.

3. Interoperability with External Systems
Care is rarely delivered by a single organization. Systems must:

- Consume and send data using national standards (e.g., HL7 FHIR, CDA)
- Work with HIEs, registries, and plan platforms
- Support APIs for third-party tools, scheduling, and patient engagement

Our open API architectures enable remote device integration, HIE connectivity, and event-based alerts across multi-system environments.

4. Real-Time or Near Real-Time Operational Dashboards
Teams managing VBC need current visibility not just monthly or quarterly reports. Systems must offer:

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- Dashboards that show cost, quality, risk, and engagement metrics
- Filters by team, condition, geography, or population segment
- Alerting or nudges when metrics drop below threshold

SpinSys-Diné platforms support HEDIS and eCQM dashboards built directly on live BDV and Bulk FHIR data streams reducing reporting lag and enabling daily action.

5. Embedded Workflows for Care Coordination
Care coordination only works if it fits into the clinical workflow. IT systems should support:

- Shared task lists, care plans, and communication logs
- Role-based views for social workers, navigators, and providers
- Reminders tied to real-time status (e.g., missed labs, open referrals)

We've deployed these in FQHCs and tribal health settings, ensuring every user has access to what they need; no more, no less.

6. Patient Engagement Platforms with Feedback Loops
VBC succeeds when patients participate. IT systems must include:

- Secure messaging, reminders, and education tools
- Ability for patients to update goals, report outcomes, or flag concerns
- Tracking of engagement and adherence over time

7. Quality and Cost Reporting Aligned with Action
Reports must be more than static PDFs. IT capabilities should allow:

- Real-time visibility into measure performance (e.g., HEDIS, MIPS)
- Drill-down into gaps by provider or site
- Links from measure scores back to underlying documentation or encounter data

SpinSys-Diné has automated these reports through real-time pipelines, enabling self-updating performance dashboards and triggering targeted outreach.

8. Consent and Privacy Management
Value-based systems rely on data exchange, but patient trust is critical. Systems must:

- Manage digital consent across use cases (e.g., data sharing, remote monitoring)
- Audit data access and usage
- Allow patients or proxies to delegate access securely

How SpinSys-Diné Can Support CMS

CMS can accelerate adoption of essential IT capabilities for value-based care by embedding them as core infrastructure requirements in APM participation. SpinSys-Diné can help CMS deliver this transformation with proven tools and real-world experience:

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- **Multiplatform Data Acquisition, Collection, and Analytics (MDACA):** We enable real-time, longitudinal patient data integration across payer and provider systems supporting proactive care planning and risk stratification across diverse populations.
- **Big Data Virtualization (BDV) and Bulk FHIR pipelines:** Our BDV and Bulk FHIR solutions power live dashboards for HEDIS, eCQMs, and other quality measures, enabling near real-time performance tracking and actionable insights for care teams.
- **AI-powered risk stratification and predictive analytics:** We have deployed AI-driven risk stratification engines in DHA and CMS MIP environments supporting early intervention, patient attribution, and dynamic care management workflows.
- **Open API architectures:** SpinSys-Diné delivers FHIR-based interoperability frameworks that enable seamless data exchange with HIEs, registries, remote devices, and third-party engagement tools.
- **Embedded care coordination workflows:** We have implemented shared task lists, care plans, and role-based views in FQHCs and tribal health settings ensuring coordinated care delivery aligned with value-based goals.
- **Automated quality and cost reporting:** SpinSys-Diné has automated quality and cost reporting pipelines—enabling drill-down views, gap closure tracking, and direct links between measure performance and underlying encounter data.
- **Consent and privacy management:** Our consent management engines, aligned with CMS, IHS, and Tribal policies, ensure ethical, secure use of health data—supporting patient trust and transparency in value-based care delivery.

SpinSys-Diné is ready to help CMS embed these essential IT capabilities into APM operations and certification—ensuring that value-based care models are supported by connected, proactive, and accountable digital infrastructure that improves outcomes for all beneficiaries.

4.0 Summary

CMS doesn't need to solve these challenges alone, but can lead by setting clear expectations, defining standards that reflect how care is delivered, and making it easier for providers, developers, and patients to participate in digital health with confidence.

SpinSys Diné is ready to support that leadership. We bring tools that already work in federal health systems, from DoD and IHS to and VA. We've stabilized legacy infrastructure, modernized data pipelines, and delivered platforms that serve frontline users providers, patients, caregivers, and analysts.

We've done this work under pressure: in tribal clinics with limited connectivity, in military hospitals facing surges, in programs where data moves across hundreds of systems, and in developer ecosystems where scalable, safe innovation must be enabled by design.

We understand what it takes to move from compliance to outcomes, and from potential to trusted, scalable adoption.

SpinSys-Diné brings a proven track record delivering trusted, scalable, AI-ready data platforms that align with CMS's strategic goals for interoperability, innovation, and equitable, value-based care.

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CMS's future depends on building a digital ecosystem that delivers on three goals:

- **Stabilize:** unified APIs, robust data governance, and scalable interoperability
- **Modernize:** AI-enabled care coordination, risk stratification, and decision support
- **Transform:** personalized, equitable digital experiences for patients and caregivers, powered by real-time engagement and feedback

SpinSys-Diné stands ready to partner with CMS to advance this vision faster and with less risk, using proven accelerators such as Big Data Virtualization (BDV), Synthetic Data Engine (SDE), GENOME, Multiplatform Data Acquisition, Collection, and Analytics (MDACA), Trusted Interoperability Patterns, and our Conversational Guidance Engine.

We can also help CMS build and scale the developer ecosystem that will drive continued innovation providing trusted infrastructure, test environments, and certification pathways that encourage safe, scalable adoption of digital health tools.

We welcome the opportunity to engage directly with CMS leadership and technical teams. We're prepared to bring in our engineers, architects, and designers not just to discuss ideas, but to design actionable pathways, deliver reference implementations, and help CMS build the trusted digital infrastructure that value-based, person-centered care requires.

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